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MC-2202

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Passport No :

LABORATORY TEST REPORT

Patient Information	Sample Information	Client/Location Information
Name : Lyubochka Svetka	Lab Id : 02232160XXXX	Client Name : Sterling Accuris Buddy
Sex/Age : Male / 41 Y 01-Feb-1982	Registration on : 20-Feb-2023 09:10	Location :
Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 11:10 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Biochemistry

Test	Result	Unit	Biological Ref. Interval
Creatinine, Serum <i>Creatinine Amidohydrolase</i>	0.83	mg/dL	0.66 - 1.25
Urea <i>Urease, Colorimetric</i>	L 18.0	mg/dL	19.3 - 43.0
Blood Urea Nitrogen <i>Calculated</i>	L 8.41	mg/dL	9.0 - 20.0
Uric Acid <i>Uricase</i>	4.90	mg/dL	3.5 - 8.5
Calcium <i>Arsenazo III</i>	9.10	mg/dL	8.4 - 10.2
SGPT <i>UV with P5P, IFCC</i>	48.0	U/L	0 - 50
SGOT <i>UV with P5P</i>	27.0	U/L	17 - 59

Electrolytes

Sodium (Na+) <i>Direct- ISE</i>	143.00	mmol/L	136 - 145
Potassium (K+) <i>Direct- ISE</i>	4.90	mmol/L	3.5 - 5.1
Chloride (Cl-) <i>Direct- ISE</i>	105.0	mmol/L	98 - 107

Dr. Purvish Darji
MD(Path)

Dr. Sanjeev Shah
MD Path

Dr. Yash Shah
MD Path

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Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 12:33 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Immunoassay

Test	Result	Unit	Biological Ref. Interval
25(OH) Vitamin D CLIA	8.98	ng/mL	Deficiency : <10 Insufficiency : 10 - 30 Sufficiency : 30 - 100 Toxicity : >100

Vitamin D is a fat soluble vitamin and exists in two main forms as cholecalciferol(vitamin D3) which is synthesized in skin from 7-dehydrocholesterol in response to sunlight exposure & Ergocalciferol(vitamin D2) present mainly in dietary sources.Both cholecalciferol & Ergocalciferol are converted to 25(OH)vitamin D in liver.

Interpretation:

Increased In

- Vitamin D intoxication
- Excessive exposure to sunlight

Decreased In

- Malabsorption
- Steatorrhea
- Dietary osteomalacia, anticonvulsant osteomalacia
- Biliary and portal cirrhosis
- Thyrotoxicosis
- Pancreatic insufficiency
- Celiac disease
- Rickets
- Alzheimer disease

Limitations:

More recently, it has become clear that receptors for vitamin D are present in a wide variety of cells and that this hormone has biologic effects extending beyond the control of mineral metabolism. Vitamin D deficiency is not clear. Levels needed to prevent rickets and osteomalacia (15 ng/mL) are lower than those that dramatically suppress parathyroid hormone levels (20–30 ng/mL). In turn, those levels are lower than levels needed to optimize intestinal calcium absorption (34 ng/mL). Neuromuscular peak performance is associated with levels approximately 38 ng/mL. A recent study states that increasing mean baseline levels from 29 to 38 ng/mL was associated with a 50% lower risk for colon cancer and levels of 52 ng/mL with a 50% reduction in the incidence of breast cancer. It is recommended to have clinical correlation with serum 25(OH)vitamin D, serum calcium, serum PTH & serum alkaline phosphatase.

Dhote.

DR. TEJASWINI DHOTE

M.D. Pathology

Dr. Sanjeev Shah

MD Path

Dr. Yash Shah

MD Path

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Sterling Accuris Pathology Laboratory

National Reference Laboratory: 101 to 109, First Floor, Sankalp Square II, Nr. Old Sharda Mandir Cross Road, B/s. Jalaram Mandir Railway Crossing, Ellisbridge, Ahmedabad, Gujarat- 380006, **Ph.: 812 813 0000**
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Sex/Age : Male / 41 Y 01-Feb-1982	Registration on : 20-Feb-2023 09:10	Location :
Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 12:04 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Immunoassay

Test	Result	Unit	Biological Ref. Interval
Vitamin B12 CLIA	L < 148	pg/mL	187 - 833

Vitamin B12 is essential in DNA synthesis, hematopoiesis, and CNS integrity.

Interpretation:

- Increased In** : Chronic granulocytic leukemia , COPD and Chronic renal failure , Leukocytosis , Liver cell damage (hepatitis, cirrhosis) , Obesity and Severe CHF , Polycythemia vera , Protein malnutrition.
- Decreased In** : Abnormalities of cobalamin transport or metabolism , Bacterial overgrowth , Crohn disease , Dietary deficiency (e.g. in vegetarians) , Diphylobothrium (fish tapeworm) infestation , Gastric or small intestine surgery , Hypochlorhydria , Inflammatory bowel diseases , Intestinal malabsorption and Intrinsic factor deficiency

Limitations:

- Drugs such as chloral hydrate increase vitamin B12 levels. On the other hand , alcohol, aminosalicic acid, anticonvulsants, ascorbic acid, cholestyramine, cimetidine, colchicines, metformin, neomycin, oral contraceptives, ranitidine, and triamterene decrease vitamin B12 levels.
- The evaluation of macrocytic anemia requires measurements of both vitamin B12 and folate levels; ideally they should be measured simultaneously.
- Specimen collection soon after blood transfusion can falsely increase vitamin B12 levels.
- Patients taking vitamin B12 supplementation may have misleading results.
- A normal serum concentration of B12 does not rule out tissue deficiency of vitamin B12. The most sensitive test for B12 deficiency at the cellular level is the assay for MMA. If clinical symptoms suggest deficiency, measurement of MMA and homocysteine should be considered, even if serum B12 concentrations are normal.

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Sex/Age : Male / 41 Y 01-Feb-1982	Registration on : 20-Feb-2023 09:10	Location :
Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 11:38 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Immunoassay

Test	Result	Unit	Biological Ref. Interval
PSA-Prostate Specific Antigen, Total	0.573	ng/mL	0 - 4

PSA is a glycoprotein that is expressed by both normal and neoplastic prostate tissue and is prostate tissue specific and not prostate cancer specific. PSA is constantly expressed in nearly all prostate cancers, although its level of expression on a per cell basis is lower than in normal prostate epithelium. The absolute value of serum PSA is useful for determining the extent of prostate cancer and assessing the response to prostate cancer treatment; its use as a screening method to detect prostate cancer is also common.

Interpretation

Increased in

- Prostate disease (Cancer, Prostatitis, Benign prostatic hyperplasia, Acute urinary retention)
- Manipulations (Cystoscopy, Needle biopsy, Radiation therapy, Indwelling catheter, Prostatic massage)
- Transurethral resection
- Prostatic ischemia

Decreased in

- Castration
- Prostatectomy
- Radiation therapy
- Ejaculation with 24 - 48 hours
- 5-alpha-reductase inhibitor reduces PSA by 50% after 6 months in men without cancer

Limitations

- PSA has been recommended by the American Cancer Society for use in conjunction with a DRE for early detection of prostate cancer starting at the age of 50 years for men with at least 10 year life expectancy
- PSA levels that are measured repeatedly over time may vary because of biologic variability where the true PSA level in a given man is different on different measurements.
- A change in PSA of >30% in man with a PSA initially below 2.0 ng/mL was likely to indicate a true change beyond normal random variation.

Dr. Purvish Darji
MD(Path)

Dr. Sanjeev Shah
MD Path

Dr. Yash Shah
MD Path

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Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 12:06 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Immunoassay

Test	Result	Unit	Biological Ref. Interval
IgE CLIA	H 492.30	IU/mL	0 - 87

IgE mediates allergic and hypersensitivity reactions. There is a significant overlap in total IgE between allergic and nonallergic individuals.

Interpretation:

- Increased In

- Atopic diseases
 - Exogenous asthma in approximately 60% of patients
 - Hay fever in approximately 30% of patients and Atopic eczema
- Influenced by type of allergen, duration of stimulation. Presence of symptoms, and hyposensitization treatment
- Parasitic diseases (e.g. ascariasis, visceral larva migrans, hookworm disease, schistosomiasis, Echinococcus infestation)
- Monoclonal IgE myeloma

- Decreased In

- Hereditary deficiencies
- Acquired immunodeficiency
- Ataxia-telangiectasis
- Non-IgE myeloma

Limitations:

- A normal level of IgE in serum does not eliminate the possibility of allergic disease.
- Serum total IgE levels for the majority of individuals with IgE-mediated disease can be expected to be elevated compared to the reference range for healthy adults. However, not all allergic patients exhibit elevated serum total IgE levels.
- Since not all atopic reactions are IgE-mediated, a total IgE result in the reference range should always be interpreted in light of other clinical observations.
- Heterophilic antibodies in human serum can react with the immunoglobulins included in the assay components causing interference with in vitro immunoassays.

Dr. Purvish Darji
MD(Path)

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MD Path

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Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Serum	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Test	Result	Unit	Biological Ref. Interval
HIV I & II Ab/Ag with P24 Ag <i>Chemiluminescence</i>	0.070	S/Co	Non Reactive : <1.0 Reactive : >1.0
Interpretation	Non Reactive		
HBsAg <i>Chemiluminescence</i>	0.290	S/Co	Non Reactive : <1.0 Reactive: >1.0
Interpretation	Non Reactive		

Additional Information:

1. A NON REACTIVE result implies that no Anti HIV-1 or HIV -2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been tested during the "WINDOW PHASE" (before the development of detectable levels of antibodies).
 2. A PROVISIONALITY REACTIVE / BORDERLINE REACTIVE result suggests possibility of HIV-1 or/and HIV-2 infection. However these results must be verified by confirmatory WESTERN BLOT / HIV PCR method before declaring the patient positive for HIV-1 or HIV-2 infection.
 3. Very high levels of IgM Antibodies or Anti-HLA ABC and DR Antibodies can give false positive reaction.
- **Pre & Post test counselling for HIV testing is responsibility of referring Physician.**

Dr. Siddharth Thummar
M.D. Pathology

Dr. Sanjeev Shah
MD Path

Dr.Yash Shah
MD Path

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Ref. Id :	Collected at : non SAWPL	Approved on : 20-Feb-2023 14:16 Status : Final
Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : EDTA Blood	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

HB Electrophoresis By HPLC

Instrument Name: BIORAD VARIANT - II Haemoglobin Testing System

Test	Result	Unit	Biological Ref. Interval
Hb A	L 84.4	%	96.8 - 97.8
Hb A2	2.8	%	2.2 - 3.2
P2 Peak	5.5	%	
P3 Peak	5.2	%	
Foetal Hb	0.3	%	0.0 - 1.0
Interpretation	Negative for typical beta thalassemia trait.		

Interpretation:

- All results have to be correlated with age and history of blood transfusion if there is history of blood transfusion in last 3 months, repeat testing after 3 month from last date of transfusion is recommended.
- In case of haemoglobinopathy, parents or family studies and counselling is advised.
- This test detects beta thalassaemia and haemoglobinopathies, DNA analysis is recommended to rule out alpha thalassaemia and silent carriers.
- Linearity range of HbF is 1-40%, However, values in excess of the reportable range have been provided for ease of interpretation.
- Mild to moderate increase in fetal haemoglobin can be seen in some acquired condition like pregnancy, megaloblastic anaemia, Throtoxicosis, Hypoxia, Chronic kidney disease, Recovering marrow, MDS, Aplastic anaemia, PNH, Medications (Hydrocurea, Erythropoietin) ect.
- P3 window-Above 10% is often indicative of either denatured forms of hemoglobins or may suggest a possibility of abnormal haemoglobin variant. Hence, repeat analysis with fresh sample or DNA studies is advised.
- P2 Window-Above 10% is indicative of either glycated haemoglobin requiring correlation with diabetic status or may suggest a possibility of abnormal haemoglobin variant further DNA studies for confirmation.

Dr. Hardik Modi
Hematopathologist (G-18097)

Dr. Sanjeev Shah
MD Path

Dr. Yash Shah
MD Path

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Patient Data

Sample ID: 022321600126
Patient ID:
Name:
Physician:
Sex:
DOB:
Comments:

Analysis Data

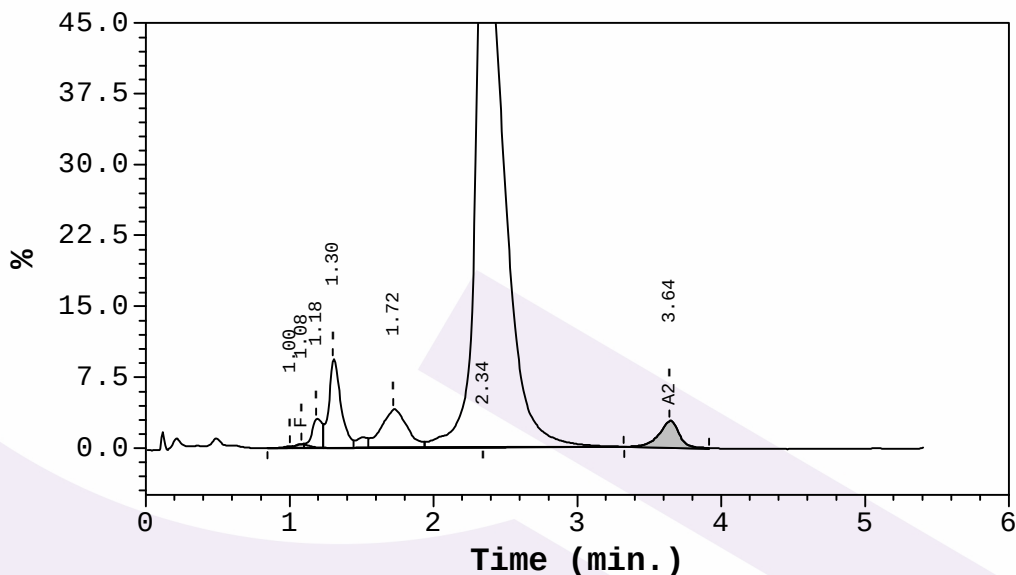
Analysis Performed: 02/20/2023 13:56:59
Injection Number: 2575
Run Number: 95
Rack ID: 0001
Tube Number: 2
Report Generated: 02/20/2023 14:04:40
Operator ID:

Peak Name	Calibrated Area %	Area %	Retention Time (min)	Peak Area
Unknown	---	0.1	1.00	2400
F	0.3	---	1.08	8321
Unknown	---	1.6	1.18	44109
P2	---	5.5	1.30	148807
P3	---	5.2	1.72	139183
A0	---	84.4	2.34	2277592
A2	2.8	---	3.64	76999

Total Area: 2,697,411

F Concentration = 0.3 %
A2 Concentration = 2.8 %

Analysis comments:





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Ref. By :	Collected on : 20-Feb-2023 08:53	Printed On : 28-Feb-2023 10:26
	Sample Type : Urine	Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Test	Result	Unit	Biological Ref. Interval
<u>Physical & Chemical (Dip strip) examination</u>			
Colour	Pale Yellow		Pale Yellow
Clarity	Clear		Clear
pH <i>Double indicator</i>	6.0		4.6 - 8.0
Specific Gravity <i>Polyelectrolyte based reaction</i>	1.030		1.005 - 1.030
Urine Glucose <i>GOD-POD</i>	Present (+)		Absent
Urine Protein <i>Protein error of indicators</i>	Absent		Absent
Bilirubin <i>Diazo reaction</i>	Absent		Absent
Urobilinogen <i>Modified Ehrlich reaction</i>	Absent		Absent
Urine Ketone <i>Nitroprusside</i>	Absent		Absent
Nitrite <i>Nitrite reaction</i>	Absent		Absent
<u>Microscopic Examination</u>			
Pus Cells	1-2		Absent
Red Cells	Nil	/hpf	0 - 2
Epithelial Cells	1-2	/hpf	.
Casts	Absent	/hpf	Absent
Crystals	Absent	/hpf	Absent
Amorphous Material	Absent		

----- End Of Report -----

DR. TEJASWINI DHOTE

M.D. Pathology

Dr. Sanjeev Shah

MD Path

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